

1. Administrative & Study Identification

Official Title: Phase III Randomized, Open-Label, Multicenter Study Evaluating Efficacy and Safety of Mosunetuzumab in Combination With Lenalidomide in Comparison to Rituximab in Combination With Lenalidomide With a Non-Randomized Single Arm US Extension of Mosunetuzumab in Combination With Lenalidomide in Patients With Follicular Lymphoma After at Least One Line of Systemic Therapy **Study Title:** A Study Evaluating the Efficacy and Safety of Mosunetuzumab in Combination With Lenalidomide in Comparison to Rituximab in Combination With Lenalidomide With a US Extension of Mosunetuzumab in Combination With Lenalidomide in Participants With Follicular Lymphoma **Short Title/Acronym:** CELESTIMO **Protocol Number:** G042909 **NCT Number:** NCT04712097 **Posting ID:** 642139422628590406 **Trial Status:** Active, Not Recruiting (Active but not currently recruiting) **Sponsor/Company Name:** Hoffmann-La Roche / Roche **Investigational Product:** Mosunetuzumab (SC) in combination with Lenalidomide (Oral) **Phase of Development:** Phase III **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy, specifically Progression-Free Survival (PFS), of mosunetuzumab plus lenalidomide versus rituximab plus lenalidomide (\$R^2\$) in patients with relapsed or refractory (R/R) follicular lymphoma (FL) who have received at least one line of prior systemic therapy. **Secondary Objectives:** To evaluate the safety, PFS as determined by the investigator, objective response rate (ORR), complete response (CR) rate, pharmacokinetics, and immunogenicity.

2.2 Background & Rationale Despite favorable outcomes with frontline therapy, the majority of patients with follicular lymphoma (FL) will ultimately relapse and eventually succumb to the disease or treatment complications. Lenalidomide is an oral immunomodulatory agent that is a rational partner for anti-CD20 therapy. This study aims to determine if the combination of the CD20xCD3 bispecific antibody mosunetuzumab with lenalidomide improves outcomes compared to the standard \$R^2\$ regimen in previously treated FL patients.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Rituximab + Lenalidomide) **Blinding:** Open-label **Randomization:** 1:1, Stratified by POD24 status (progression within 24 months), number of prior lines of therapy (1 vs. ≥ 2), and refractoriness to anti-CD20 therapy.

3.2 Planned Enrollment & Duration Target N: 478 participants **Number of Sites:** ~150 participating centers globally (United States, Brazil, Asia, Europe, and Australia) **Estimated Study Start:** 2021 **Estimated Primary Completion:** Spring 2023 (Accrual completion)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0, 1, or 2. 2. Histologically documented CD20+ FL (Grades 1-3a) requiring systemic therapy assessed by investigator based on tumor size and/or Groupe d'Etude des Lymphomes Folliculaires (GELF) criteria. 3. Received at least one prior systemic lymphoma therapy, which included prior immunotherapy or chemoimmunotherapy. 4. Availability of a representative tumor specimen (core-needle, excisional or incisional biopsy; cytological/fine-needle aspiration samples are not acceptable). 5. Adequate hematologic function. 6. Agreement to comply with all local requirements of the lenalidomide risk minimization plan, including the global pregnancy prevention program. 7. Strict contraception protocols for women of childbearing potential and men during the study and for specified periods post-treatment.

4.2 Exclusion Criteria 1. Grade 3b FL, any history of disease transformation, and/or diffuse-large B cell lymphoma (DLBCL). 2. Documented refractoriness to lenalidomide (no response or relapse within 6 months of therapy). 3. Active or history of CNS lymphoma or leptomeningeal infiltration. 4. Prior therapy limitations: Lenalidomide exposure within 12 months; CAR-T therapy within 30 days; radioimmunoconjugate within 12 weeks; monoclonal antibody/ADC within 4 weeks. 5. Clinically significant unresolved toxicity from prior treatment ($>$ Grade 1 per NCI CTCAE v5.0). 6. Treatment with systemic immunosuppressive medications (e.g., $>$ 20 mg prednisone) within 2 weeks prior to Cycle 1 Day 1. 7. History of solid organ transplantation or prior allogeneic stem cell transplantation. 8. Known severe hypersensitivity to humanized/chimeric/murine antibodies, CHO cells, or product formulations (e.g., mannitol). 9. History of interstitial lung disease, pneumonitis, or erythema multiforme/blistering following prior immunomodulatory derivatives. 10. Active bacterial, viral, fungal, or other infections (including known HIV, active Hep B/C, chronic active EBV, and progressive multifocal leukoencephalopathy [PML]). 11. Live, attenuated vaccine

administration within 4 weeks of the first dose. 12. Active autoimmune disease requiring treatment, significant cardiovascular disease, or major surgical procedures within 28 days. 13. Pregnant or lactating women.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Arm A receives Subcutaneous (SC) Mosunetuzumab + Oral Lenalidomide. Arm B receives IV/SC Rituximab (Trade names: Rituxan, Riabni, Ruxience, Truxima, Rituxan Hycela) + Oral Lenalidomide. Treatment consists of 12 induction cycles. Cycle 1 is 21 days; Cycle 2 onwards are 28 days. Participants in Arm A achieving a complete or partial metabolic response (CMR/PMR) after 12 cycles receive maintenance therapy with SC mosunetuzumab every 8 weeks (Q8W) for 9 additional cycles. **Route of Administration:** Subcutaneous (Mosunetuzumab), Oral (Lenalidomide), IV/SC (Rituximab). **Duration of Treatment:** Up to 21 cycles total (12 induction cycles + 9 maintenance cycles).

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing disease symptoms or treatment-related AEs (e.g., tocilizumab for cytokine release syndrome if applicable). **Prohibited:** Other investigational agents or concurrent systemic anti-lymphoma therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, PET scan/Tumor Biopsy, Baseline Labs, Contraception counseling
Baseline	Cycle 1, Day 1	Randomization, First Dose (21-day cycle length)
Induction	Cycles 2-12	(28-day cycles) Safety Labs, Efficacy Assessment (PET/CT), IP Administration
Maintenance	Post-Cycle 12	Q8W SC Mosunetuzumab administration (for Arm A responders only) up to 9 cycles
End of Study		Follow-up Final Vital Signs, Exit Interview, Disease Progression / Overall Survival monitoring

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS) according to 2014 Lugano Response Criteria. **Measurement Method:** Assessed by an Independent Review Committee (IRC).

7.2 Secondary & Exploratory Endpoints 1. Progression-Free Survival (PFS) as Determined by the Investigator. 2. Complete Response (CR) Rate and Objective Response Rate (ORR). 3. Pharmacokinetics and immunogenicity of mosunetuzumab. 4. Safety and tolerability profiles.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard continuous collection, particularly focusing on known side effects of mosunetuzumab (e.g., cytokine release syndrome), lenalidomide, and rituximab. **Serious Adverse Events (SAEs):** Reported within standard 24-hour safety timelines. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (iDMC) to review safety and efficacy signals.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy analysis (PFS). Safety Set for all AE/SAE evaluations. **Sample Size Justification:** $N=478$ adequately powered to detect statistically significant improvements in PFS between the investigational arm and the active control arm. **Handling of Missing Data:** Standard survival analysis principles; right-censored at the last evaluable disease assessment for PFS.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular clinical site monitoring visits for source data verification and regulatory compliance. **Audit Trail:** Strict documentation of eCRFs, tumor response central reading (by IRC), and data clarification forms.

11. Study Identifiers & Governance

Primary ID: {{G042909}} **Posting ID:** {{642139422628590406}}
Registry Number: {{NCT04712097}} **Sponsor/Lead Organization:** {{Roche}} **Study Title:** {{A Study Evaluating the Efficacy and Safety of Mosunetuzumab in Combination With Lenalidomide in Comparison to Rituximab in Combination With Lenalidomide With a US Extension of Mosunetuzumab in Combination With Lenalidomide in Participants With Follicular Lymphoma}} **Official Regulatory Title:** {{Phase III Randomized, Open-Label, Multicenter Study Evaluating Efficacy and Safety of Mosunetuzumab in Combination With Lenalidomide in Comparison to Rituximab in Combination With Lenalidomide With a Non-Randomized Single Arm US Extension of Mosunetuzumab in Combination With Lenalidomide in Patients With Follicular Lymphoma After at Least One Line of Systemic Therapy}}

12. Clinical Framework (Follicular Lymphoma Specific)

Primary Condition: {{Relapsed or Refractory Follicular Lymphoma (FL)}} **Diagnosis Threshold:** {{CD20+ Grade 1-3a FL requiring systemic therapy}} **Medical Methodology:** {{Bispecific Antibody and Immunomodulator Therapy}} **Optimization Target:** {{Progression-Free Survival (PFS) prolongation}}

13. Study Procedures & Methodology

Management Pathway: {{Mosunetuzumab + Lenalidomide vs. Rituximab + Lenalidomide}} **Education Protocol (HCP):** {{Protocol-specific safety (CRS) and administration training}} **Education Protocol (Patient):** {{Contraception requirements and AE monitoring education}} **Quality Audit Mechanism:** {{Independent Review Committee (IRC) efficacy assessment}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to 21 cycles (12 cycles induction + 9 cycles maintenance)}} **Onsite Visit Cadence:** {{Every 21-28 days during induction, Q8W during maintenance}} **Remote Monitoring Frequency:** {{Per protocol standard monitoring plan}} **Checklist Review Interval:** {{Every treatment cycle}}

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				